



LABORATORY REPORT



Name : Mrs PADMABEN SHAH	Sex/Age : Female / 76 Years	Case ID : 51200110808
Ref. By : DR. HETAN SHAH	Dis. At :	Pt. ID : 3440020
Bill. Loc. :		Pt. Loc. :
Reg Date and Time : 10-Dec-2025 11:54	Sample Type :	Mobile No. : 7819811953
Sample Date and Time :	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time :	Acc. Remarks :	Ref Id2 :

Abnormal Result(s) Summary

Test Name	Result Value	Unit	Reference Range
Glyco Hemoglobin (HbA1c)			
HbA1c	6.00	% of total Hb	<5.7: Normal 5.7-6.4: Prediabetes ≥6.5: Diabetes
Haemogram (CBC)			
Eosinophil	7.6	%	1.00 - 6.00
Eosinophil	751	/μL	20.00 - 500.00
Renal Function Test			
Uric Acid	7.29	mg/dL	2.6 - 6.0
Urine Examination			
Blood	++		Negative
RBC (Isomorphic)	21	/HPF	Nil
ESR	93	mm after 1hr	3 - 30

Abnormal Result(s) Summary End

Note:

This summary highlights abnormal and trend results extracted from the report for quick reference. It is for informational purposes only and does not require a doctor's signature.

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

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MC-6136

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Reg Date and Time : 10-Dec-2025 11:54	Sample Type : Whole Blood EDTA	Mobile No. : 7819811953
Sample Date and Time : 10-Dec-2025 10:22	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 10-Dec-2025 12:38	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
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HAEMOGRAM REPORT

HB AND INDICES

Haemoglobin	13.4	G%	12.00 - 15.00	
RBC (Electrical Impedance)	4.51	millions/cum m	3.80 - 4.80	
PCV(Calc)	42.39	%	36.00 - 46.00	
MCV (RBC histogram)	94.0	fL	83.00 - 101.00	
MCH (Calc)	29.8	pg	27.00 - 32.00	
MCHC (Calc)	31.6	gm/dL	31.50 - 34.50	
RDW (RBC histogram)	15.10	%	11.00 - 16.00	

TOTAL AND DIFFERENTIAL WBC COUNT (Flowcytometry)

Total WBC Count	9880	/μL	4000.00 - 10000.00	
Neutrophil	65.9	%	40.00 - 70.00	
Lymphocyte	21.0	%	20.00 - 40.00	
Eosinophil	H 7.6	%	1.00 - 6.00	
Monocytes	5.2	%	2.00 - 10.00	
Basophil	0.3	%	0.00 - 2.00	
Neutrophil	6511	/μL	2000.00 - 7000.00	
Lymphocyte	2075	/μL	1000.00 - 3000.00	
Eosinophil	H 751	/μL	20.00 - 500.00	
Monocyte	514	/μL	200.00 - 1000.00	
Basophil	30	/μL	0.00 - 100.00	
Neut/Lympho Ratio (NLR) CALC	3.14		0.78 - 3.53	

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Harveen

Dr Harveen Bhusari

MD Pathology (G-62229)

Pavan

Dr. Pavan Dave

DCP, DNB (PATH) G-22473

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PLATELET COUNT (Optical)

Platelet Count	278000	/ μ L	150000.00 - 410000.00
MPV	10.50	fL	6.5 - 12
PDW	16.0	fL	15 - 17

SMEAR STUDY

RBC Morphology	Normocytic Normochromic RBCs.
WBC Morphology	Eosinophilia
Platelet	Platelets are adequate in number.
Parasite	Malarial Parasite not seen on smear.
Morphological Impression	-
ESR <i>Photometrical capillary stopped flow kinetic analysis</i>	H 93 mm after 1hr 3 - 30

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Reg Date and Time : 10-Dec-2025 11:54	Sample Type : Plasma Fluoride F,Whole Blood EDTA	Mobile No. : 7819811953
Sample Date and Time : 10-Dec-2025 10:22	Sample Coll. By : NSRL	Ref Id1 :
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TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
Plasma Glucose - F <i>Photometric,Hexokinase</i>	92.28	mg/dL	70.0 - 100	
<u>Glycated Haemoglobin Estimation</u>				
HbA1c <i>HPLC</i>	H 6.00	% of total Hb	<5.7: Normal 5.7-6.4: Prediabetes >=6.5: Diabetes	
Estimated Avg Glucose (3 Mths) <i>Calculated</i>	125.50	mg/dL	Not available	

Interpretation :

HbA1C level reflects the mean glucose concentration over previous 8-12 weeks and provides better indication of long term glycemic control.

Levels of HbA1C may be low as result of shortened RBC life span in case of hemolytic anemia.

Increased HbA1C values may be found in patients with polycythemia or post splenectomy patients.

Patients with Homozygous forms of rare variant Hb(CC,SS,EE,SC) HbA1c can not be quantitated as there is no HbA.

In such circumstances glycemic control can be monitored using plasma glucose levels or serum Fructosamine.

The A1c target should be individualized based on numerous factors, such as age, life expectancy,comorbid conditions, duration of diabetes, risk of hypoglycemia or adverse consequences from hypoglycemia, patient motivation and adherence.

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Report Date and Time : 10-Dec-2025 13:11	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
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BIOCHEMICAL INVESTIGATIONS

Renal Function Test

Urea <i>Urease</i>	32.71	mg/dL	20.97 - 43.01
Creatinine <i>Jaffe - Kinetic</i>	0.98	mg/dL	0.5 - 1.2
Uric Acid <i>Uricase</i>	H 7.29	mg/dL	2.6 - 6.0
Sodium <i>ISE</i>	144.45	mmol/L	136 - 145
Potassium <i>ISE</i>	4.88	mmol/L	3.5 - 5.1
Chloride <i>ISE</i>	101.98	mmol/L	98 - 107

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BIOCHEMICAL INVESTIGATIONS

Liver Function Test

S.G.P.T. NADH (Without P-5-P)	13.13	U/L	< 34
S.G.O.T. NADH (Without P-5-P)	18.23	U/L	5.0 - 34.0
Alkaline Phosphatase Enzymatic, PNPP-AMP	106.65	U/L	46.0 - 122.0
Gamma Glutamyl Transferase Multipoint Rate-L-y-Glytamyl-p-nitroanilide	29.37	U/L	0 - 38
Proteins (Total) Biuret	7.50	gm/dL	6.20 - 8.10
Albumin (BCG)	4.56	gm/dL	3.2 - 4.6
Globulin	2.94	gm/dL	2 - 4.1
A/G Ratio Calculated	1.55		1.0 - 2.1
Bilirubin Total Colorimetric Diazo Method	0.59	mg/dL	0.3 - 1.2
Bilirubin Conjugated Colorimetric Diazo Method	0.27	mg/dL	0 - 0.50
Bilirubin Unconjugated	0.32	mg/dL	0 - 0.8

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TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
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BIOCHEMICAL INVESTIGATIONS

Lipid Profile

Cholesterol <i>Enzymatic</i>	123.89	mg/dL	110 - 200
HDL Cholesterol <i>Accelerator Selective Detergent</i>	52.02	mg/dL	40 - 60
Triglyceride <i>Arsenazo - Colorimetric</i>	125.91	mg/dL	<150
VLDL	25.18	mg/dL	10 - 40
Chol/HDL <i>Calculated</i>	2.38		0 - 4.1
LDL Cholesterol <i>Calculated</i>	46.69	mg/dL	0.00 - 100.00

NEW ATP III GUIDELINES (MAY 2001), MODIFICATION OF NCEP

LDL CHOLESTEROL	CHOLESTEROL	HDL CHOLESTEROL	TRIGLYCERIDES
Optimal<100	Desirable<200	Low<40	Normal<150
Near Optimal 100-129	Border Line 200-239	High >60	Border High 150-199
Borderline 130-159	High >240	-	High 200-499
High 160-189	-	-	-

- LDL Cholesterol level is primary goal for treatment and varies with risk category and assesment
- For LDL Cholesterol level Please consider direct LDL value
Risk assesment from HDL and Triglyceride has been revised. Also LDL goals have changed.
- Detail test interpretation available from the lab
- All tests are done according to NCEP guidelines and with FDA approved kits.
- LDL Cholesterol level is primary goal for treatment and varies with risk category and assesment

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[Signature]

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BIOCHEMICAL INVESTIGATIONS

Skeletal Profile

Calcium OCPC	9.35	mg/dL	8.4 - 10.2	
Phosphorus Inorganic Phosphomolybdate	3.60	mg/dL	2.3 - 4.7	

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Reg Date and Time : 10-Dec-2025 11:54	Sample Type : Spot Urine	Mobile No. : 7819811953
Sample Date and Time : 10-Dec-2025 19:47	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 10-Dec-2025 20:03	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BRI	TEST	RESULTS	UNIT	BRI
URINE EXAMINATION (STRIP METHOD AND AUTOMATED IMAGE EVALUATION)							
<u>Physical and Chemical examination</u>							
Colour	Yellow			Glucose	Negative		Negative
Appearance	Clear			Ketone Bodies Urine	Negative		Negative
Sp.Gravity	1.008		1.003 - 1.035	Urobilinogen	Negative		Negative
pH	7.00		4.6 - 8	Bilirubin	Negative		Negative
Leucocytes (ESTERASE)	Negative		Negative	Nitrite	Negative		Negative
Protein	Negative		Negative	Blood	++		Negative
<u>Automated Microscopy</u>							
Leucocyte	2	/HPF	Nil	Granular Cast	Nil	/HPF	Nil
Leucocyte Clumps	Nil	/HPF		RBC cast	Nil	/HPF	Nil
RBC (Isomorphic)	21	/HPF	Nil	WBC Cast	Nil	/HPF	Nil
RBC (Dysmorphic)	Nil	/HPF	Nil	Fatty Cast	Nil	/HPF	Nil
<u>Epithelial cells</u>				Mixed Cast	Nil	/HPF	Nil
Squamous EC	Present +	/HPF	Present	<u>Crystals</u>			
NonSquamous EC	Nil	/HPF		Crystals	Nil	/HPF	Nil
Transitional EC	Nil	/HPF		Calcium oxalate Monohydrate	Nil	/HPF	Nil
Renal Tubular EC	Nil	/HPF		Calcium oxalate Dihydrate	Nil	/HPF	Nil
<u>Bacteria</u>				Triple phosphate	Nil	/HPF	Nil
Bacteria	Nil	/μL	Nil	Uric acid	Nil	/HPF	Nil
Bacteria Rods	Nil	/μL	Nil	Leucine	Nil	/HPF	Nil
Bacteria Cocci	Nil	/μL	Nil	Amorphous	2	/HPF	Nil
Yeast	Nil	/μL	Nil	Trichomonas	Nil	/μL	Nil
<u>Cast</u>							
Hyaline Cast	Nil	/HPF	Nil				
Pathological Cast	Nil	/HPF	Nil				
Hyaline granular cast	Nil	/HPF					

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Dr. Shachi Shah

M.D. (Pathologist) G-37179

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----- End Of Report -----

For test performed on specimens received or collected from non-NSRL locations, it is presumed that the specimen belongs to the patient named or identified as labeled on the container/test request and such verification has been carried out at the point generation of the said specimen by the sender. NSRL will be responsible Only for the analytical part of test carried out. All other responsibility will be of referring Laboratory.

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